

PERFORMANCE WORK STATEMENT

1. PURPOSE

- 1.1. San Francisco Veterans Affairs Health Care System is seeking a Contractor to provide residential programming and services as part of its Health Care for Homeless Veterans (HCHV) Low Demand Safe Haven Program. The Contractor shall offer community-based, short-term residential services to eligible Veterans who need immediate housing placement in conjunction with sufficient supportive services to support stabilization in the community and transition to permanent housing. This residential programming shall be offered in the City and County of San Francisco. This contract shall cover up to a total of five years, which includes one 12-month Base Year and four additional 12-month Ordering Periods.

2. BACKGROUND

- 2.1. Health Care for Homeless Veterans is an essential and critical part of Veterans Health Administration (VHA), providing a gateway to VHA and community-based supportive services for eligible Veterans who are homeless. HCHV programs were developed from the original Homeless Chronically Mentally Ill (HCMI) Program, a 6-month pilot project, established on February 12, 1987. The central goal of HCHV programs is to reduce homelessness among Veterans by conducting street outreach to Veterans who are homeless or experiencing housing instability and need case management assistance to end their homelessness and connect to community-based and Veterans Affairs (VA)-supported housing services, VA health care and other supportive services. In addition to outreach services, HCHV programs provide care, treatment, and rehabilitative services, including case management and therapeutic transitional housing assistance by contracting with community providers.
- 2.2. HCHV programs typically are the first to contact homeless Veterans, and they frequently serve as the entry point into VA's system of care. Therefore, these programs provide an outreach opportunity to assist and offer homeless Veterans an "Open Door" to prevention and supportive services.
- 2.3. Federal law provides that VA medical facilities may establish contracts with community agencies to provide residential care to include therapeutic services and treatment to eligible Veterans. The Veteran must be enrolled in the VA health care system or eligible for VA health care to be eligible for services. Since its inception, the number of HCHV Contract Residential Services programs has increased dramatically, requiring uniform inspection and oversight policies to ensure that these programs offer robust services and promote an environment that maintains the wellbeing and safety of the Veterans enrolled.

2.4. In July 2010, under the leadership and direction of the VA National Center on Homelessness among Veterans, VA Low Demand Safe Haven (LDSH aka “Safe Haven”) Programs were developed and implemented as evidence-based practice that serves as an alternative to more traditional programs for homeless Veterans. Safe Haven Programs are community-based early recovery models of care that incorporate Harm Reduction principles and practices into supportive interim housing for hard-to-reach, hard-to-engage chronically homeless Veterans with severe mental illness and/or substance use disorders. Safe Havens were created to meet the gap in access and availability for those who would otherwise have difficulty succeeding in other VA interim housing programs.

3. PROGRAM DESCRIPTION

3.1. HCHV Low Demand Safe Haven (LDSH) programs are 24-hour staffed short-term residences with private or semi-private accommodations that target the population of hard-to-reach, chronically homeless Veterans with mental illness, with substance use disorder (SUD), and/or when traditional residential treatment programs do not meet a Veteran’s needs. The low-demand or non-intrusive environment is designed to establish trust and motivate homeless Veterans to seek needed treatment services and transitional and permanent housing options. Lengths of stay in LDSH programs typically range between 4 to 6 months with the option to extend based on clinical need.

4. HCHV LDSH PROGRAM CHARACTERISTICS AND CAPABILITY REQUIREMENTS

4.1. Offers emergency housing and supportive services within a community-based facility located within the City and County of San Francisco, CA with easy access by public transportation to San Francisco VA Downtown Clinic (401 3rd Street, San Francisco, CA 94107) and/or San Francisco VA Medical Center (4150 Clement Street, San Francisco, CA 94121).

4.2. Covers up to a total of five years, that includes one 12-month Base Year and four additional 12-month Ordering Periods.

4.3. Has capacity to serve up to 22 Veterans.

4.4. Targets and prioritizes:

4.4.1. Chronically homeless Veterans with severe mental illness and/or substance use problems who are in the early stages of recovery.

4.4.2. Veterans who have not been successful in traditional housing or treatment programs.

4.4.3. Veterans who cannot, or will not, adhere to the rules of a traditional homeless services program and/or who cannot, or will not, abstain from alcohol or drug use.

- 4.5.** Reduces barriers to immediate placement and accommodates same day admissions when there is a vacancy in the program.
- 4.6.** Does not deny entry to HCHV LDSH Program based solely upon:
- 4.6.1.** Length of abstinence from alcohol or other non-prescribed substances
 - 4.6.2.** Number of previous treatment episodes
 - 4.6.3.** Time interval since the last HCHV LDSH program entry
 - 4.6.4.** Use of prescribed controlled substances including, but not limited to, prescribed FDA-approved medications such as methadone or buprenorphine
 - 4.6.5.** Legal history including legal requirement to register as a sex offender
 - 4.6.6.** Veteran's total income
 - 4.6.7.** Veteran's student or vocational status
 - 4.6.8.** Pre-existing diagnosis of mental illness, substance use disorder or other comorbid conditions
 - 4.6.9.** Veteran's willingness to engage in specific treatment(s) at time of referral
 - 4.6.10.** Veterans having a Patient Record Flag (PRF)
- 4.7.** Considers each of the special circumstances above during the screening process and makes an admissions determination based on whether the individual Veteran's needs can be met while maintaining the program's safety, security, and integrity.
- 4.8.** Provides safe and secure housing that is available to enrolled Veterans 24 hours per day, seven (7) days per week and includes:
- 4.8.1.** One bed for each enrolled Veteran.
 - 4.8.1.1.** Private or semi-private accommodations are required.
 - 4.8.2.** Access to shared or private toileting and bathing facilities.
 - 4.8.3.** Personal storage space such as a dresser, storage locker, or designated locked and secure space that shall accommodate two bags of personal belongings.
 - 4.8.3.1.** Contractor shall allow Veteran to store personal belongings for at least 72 business hours after formal HCHV exit from program.
 - 4.8.3.2.** If a Veteran abandons their belongings at the program site, Contractor shall make at least three documented attempts to contact the Veteran and VA Liaison or Designee before disposing of Veteran's belongings.
 - 4.8.4.** A means for securely and properly storing all medications and other medical equipment brought into the program for Veteran use.
 - 4.8.4.1.** Contractor shall ensure that there are appropriate policies and procedures that support the safe storage of medications for enrolled Veterans.

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4.10. Provides wraparound case management, care coordination, and supportive services individualized to each enrolled Veteran's needs including, but not limited to:

4.10.1. Assistance with attaining permanent housing.

4.10.2. Linkage to health care.

4.10.3. Support for mental health stabilization and moderation of substance misuse.

4.10.4. Vocational assistance including mentoring, coaching, linkage to job training and job placement

4.10.5. Assistance with increasing income, linkage to entitlements and budgeting.

4.10.6. Independent living skills to maintain permanent housing.

4.10.7. Social and recreational activities.

5. LOW DEMAND SAFE HAVEN CORE VALUES

5.1. The Low Demand Safe Haven environment of care:

5.1.1. Does not require abstinence or compliance with treatment as a condition of admission or continued stay.

5.1.2. Provides safe and secure housing and supportive services within a trauma-informed environment of care.

5.1.3. Is as non-intrusive as possible, i.e., demands are kept to a minimum.

5.1.3.1. However, ***low demand is not no demand*** but rather ***minimal demands with high expectations*** which include:

5.1.3.1.1. Privacy for the enrolled Veterans.

5.1.3.1.2. Expectation that Veterans will transition to permanent housing.

5.1.3.1.3. Expectation that Veterans will engage with their case managers in developing a housing plan.

5.1.3.1.4. Other treatment and programming, including mental health and substance use treatment, is optional but highly encouraged based on individual need and interest.

5.1.3.1.5. Engagement in care may look different for each Veteran enrolled in the program (see 9.5.4. for more detail).

5.1.4. Engages Veterans using harm reduction strategies with a primary focus on attaining and maintaining housing.

5.1.5. Is based on acceptance that not all mental health and substance use problems can "be fixed".

5.1.6. Uses harm reduction strategies that support Veterans in minimizing the health impacts and social effects of risky behaviors.

5.1.7. Has house rules that are simple, easily understood, kept to a minimum, and focus on staff and resident safety, such as:

5.1.7.1. No violence or threats of violence

- 7.5.** Failure to establish eligibility prior to admission and/or within 24 business hours if/when VA staff are unavailable to provide verification may result in denial of payment for services provided.
- 7.6.** A list of authorized VA ordering personnel including primary identified VA Liaison shall be made available to Contractor upon award of the contract.
 - 7.6.1.** VA employees may be added or deleted from this list during the term of the contract at the discretion of VA.
 - 7.6.2.** Contractor shall not deviate from the list of individuals authorized to approve admissions without an updated list.

8. LOW DEMAND SAFE HAVEN ADMISSIONS PRACTICES

- 8.1.** Contractor shall work to reduce barriers to admission and demonstrate flexibility with admission processes by:
 - 8.1.1.** Actively reducing or eliminating individual level barriers such as requirements for treatment compliance.
 - 8.1.2.** Actively reducing or eliminating system level barriers such as unnecessary delays between screening and admission and “housing readiness” requirements.
 - 8.1.3.** Allowing trial visits by Veterans to allow Veteran to determine if the program is a good fit.
 - 8.1.4.** Assisting Veterans with admission forms and VA program eligibility determination when needed.
 - 8.1.5.** Accepting referrals and admissions throughout the day and, at a minimum, between Monday-Friday, 8AM-8PM, when there is a vacant bed available.
 - 8.1.5.1.** Ability to accept admissions up to 24 hours per day, seven (7) days per week, including weekends and holidays, is highly preferred.
 - 8.1.6.** Accommodating same-day admissions when there are vacancies in the program.
 - 8.1.6.1.** Admissions decisions shall be made on the same day of referral for admission when there is a program vacancy and no later than 24 hours after receipt of referral if the program does not have any immediate openings.
 - 8.1.6.1.1.** Notification of all admissions decisions, any delay in admissions decisions as described in 8.1.6.1., and reasons for denial of any referral shall be provided in writing to VA Liaison, COR, and/or designee.
 - 8.1.6.2.** Contractor shall make reasonable efforts to admit a Veteran immediately when there is a vacancy in the program, especially prior to a weekend or holiday if/when there is no option to admit over the weekend or holiday and Veteran has no alternative to staying on the streets.
 - 8.1.6.2.1.** Vacant units within program shall be turned over and made available for new admissions immediately.

8.1.6.2.2. If Contractor is unable to accommodate a same-day admission when there is a vacancy in the program, then Contractor shall provide written explanation with reason for delay to VA Liaison, COR, and/or Designee.

8.1.7. Accepting program referrals from VA, other community partners, and/or self-referral by Veterans that include a verbal and/or written description of the referred Veteran's current clinical presentation, current psychosocial circumstances including living situation, and risk assessment.

8.1.7.1. Admissions decisions are not contingent upon Contractor receipt of a comprehensive psychosocial assessment or Veteran's intent to enroll in specific treatment(s) at time of referral.

8.1.7.2. It is understood that Veterans to be cared for under this contract will require care and services beyond room and board and may need more intensive case management than the general population.

8.1.7.3. Level of intensity of services will be part of an ongoing discussion with VA Liaison.

8.1.7.4. Contractor shall maintain effective communication with VA regarding any issues covered under this agreement.

8.1.7.5. Admissions process is subject to VA Liaison review and adjustment at any time to ensure program accessibility for Veterans.

8.1.8. Supporting Veterans in follow through with obtaining tuberculosis (TB) clearance within 10 business days of admission if TB clearance results are not available at time of referral.

8.1.8.1. Veterans shall be screened for TB via PPD or QuantiFERON or chest x-ray or other medically acceptable means of screening for TB.

8.1.8.2. While results may not be immediately available, Veterans shall be admitted while results are pending unless they are exhibiting any signs or symptoms of active TB.

9. VETERAN CLINICAL SERVICES

9.1. OCCUPANCY

9.1.1. Contractor shall be responsible for ensuring that a minimally acceptable level of 80% occupancy (90-100% preferred) of HCHV LDSH-funded beds is maintained through independent outreach efforts and through collaboration with VA.

9.1.2. Contractor shall be knowledgeable about best practices for outreach generally accepted in the community and shall be responsible for engaging in outreach in the community, including collaboration with SFVAHCS providers, for referrals at least

twice monthly, to achieve a satisfactory occupancy rate when occupancy does not meet the minimally acceptable level.

- 9.1.3.** Rate of occupancy may result in increase or decrease in program size in subsequent Ordering Periods depending on Government need and funding availability.

9.2. ACHIEVING PERMANENT HOUSING is a primary goal of the HCHV LDSH program and plans for transition from HCHV LDSH to permanent housing placements which may include transition to other residential treatment and/or transitional housing as an interim step for some Veterans, must be clearly reflected in each Veteran's Individualized Service Plan and in weekly case management notes.

- 9.2.1.** Should a Veteran refuse to engage with Contractor staff regarding plans for permanent housing transition, motivational interviewing and other therapeutic techniques shall be used to address Veteran ambivalence.

- 9.2.2.** All efforts to engage Veterans regarding housing plans, options, and resources shall be clearly documented.

- 9.2.3.** Contractor staff must provide direct and ongoing assistance to Veterans in developing permanent housing plans and accessing housing resources and may include, but is not limited to:

- 9.2.3.1.** Assistance with obtaining and reviewing free credit reports.
 - 9.2.3.2.** Identifying housing strengths and barriers.
 - 9.2.3.3.** Assistance with creating a Tenant Portfolio that includes gathering all relevant documents required for the rental process such as sample rental application, income verification documents, identification (e.g., birth certificate, state ID, social security card, DD214) and subsidy applications.
 - 9.2.3.4.** Reviewing/searching for affordable housing options together.
 - 9.2.3.5.** Assisting with completion of rental applications.
 - 9.2.3.6.** Direct communication and advocacy with prospective landlords, housing agencies, and HUD-VASH staff.

- 9.2.4.** Contractor staff are expected to participate actively in Veteran's search for housing and to document those efforts and progress made in weekly case management notes.

9.3. CARE PLANNING

- 9.3.1.** Contractor shall engage the Veteran in a collaborative assessment of service needs with input from the Veteran and the VA Liaison or designee and create an initial Individualized Service Plan (ISP) to address those needs within 14 days of admission.
- 9.3.2.** ISPs must include goals and objectives for achieving stable housing.
- 9.3.3.** ISPs shall include an assessment of strengths and barriers to stable housing, specific services to be provided including duration and outcomes, documentation of referrals and benefits to be achieved as a result of program participation.
- 9.3.4.** ISPs shall be reviewed and updated within 60 days of admission with the Veteran and the VA Liaison or designee and more frequently as needed throughout each individual Veteran's episode of care.
- 9.3.5.** Contractor staff shall meet with VA Liaison or designee at least once every 30 days to provide an update on case management progress made and must work cooperatively with VA Liaison or designee to coordinate services.

9.4. CASE MANAGEMENT

- 9.4.1.** Contractor shall provide individual case management meetings at least once per week that focus on each Veteran's individualized needs as identified through the care planning process which may include, but is not limited to:
 - 9.4.1.1.** Permanent housing search (refer to Section 9.2.).
 - 9.4.1.2.** Increasing income through referral to all potential financial resources relevant to individual needs such as:
 - 9.4.1.2.1.** SSI/SSDI
 - 9.4.1.2.2.** VA pensions and service-connected compensation claims.
 - 9.4.1.2.3.** General Assistance
 - 9.4.1.2.4.** Supplemental Nutrition Assistance Program (SNAP)
 - 9.4.1.2.5.** Medi-Cal or Medicare
 - 9.4.1.2.6.** Employment and vocational training
 - 9.4.1.2.7.** Self-care skills, independent living skills, adaptive coping skills.
 - 9.4.1.2.8.** Any necessary coordination of care provided by VA and community-based medical, mental health, and/or substance use treatment providers.
 - 9.4.1.2.9.** Ongoing discharge planning that includes linkage to community resources for long-term housing success such as In-Home Supportive Services, Meals on Wheels, or Adult Day Health Care.

9.5. THERAPEUTIC AND REHABILITATIVE SERVICES INCLUDING MENTAL HEALTH AND SUBSTANCE USE DISORDER TREATMENT

9.5.1. Contractor shall be expected to provide treatment and services using evidence-based practices that support Veterans in gaining and applying knowledge of the recovery process to their lives and to provide referrals for additional treatment and aftercare supports as needed to promote and facilitate each Veteran's recovery efforts and successful transition to permanent housing.

9.5.2. Treatment shall be defined as services which are needed for the specific clinical presentation of each enrolled Veteran including, but not limited to:

- 9.5.2.1.** VA-based services, and/or
- 9.5.2.2.** Community-based care, and/or
- 9.5.2.3.** Services provided by Contractor at the program site.

9.5.3. Services may include, but are not limited to:

- 9.5.3.1.** Clinical case management.
- 9.5.3.2.** Individual and group counseling.
- 9.5.3.3.** Structured group activities such as 12-step meetings, vocational training, independent living skills such as budgeting, cooking or hygiene, and prosocial outings and activities.
- 9.5.3.4.** Regularly scheduled (daily or weekly) community meetings that offer a forum for enrolled Veterans to discuss problems between members of the community, air grievances about the program, and resolve any issues before they escalate into more serious conflicts.

9.5.4. Working towards engagement in the types of treatment and services that meet the individualized needs of each enrolled Veteran shall be integral to the program's care planning process.

- 9.5.4.1.** Engagement in services and the extent to which assertive engagement is needed may look different from one Veteran to another.
- 9.5.4.2.** Veterans are expected to develop a housing plan and to maintain communication with their case managers around matters relating to their admission, stay, and treatment.
- 9.5.4.3.** Veterans are encouraged, but not required, to engage meaningfully in the relevant programming available to them.
- 9.5.4.4.** Contractor staff are expected to actively engage Veterans in services from time of admission to time of discharge through outreach efforts, rapport-building, motivational interviewing, and Veteran-centered SMART (Specific, Measurable, Achievable, Relevant, and Time-Bound) goals.

9.5.5. Contractor shall ensure that a completed and signed VA Form 10-5345 Request for and Authorization to Release Health Information (ROI) is obtained for every Veteran admitted to the HCHV LDSH program.

9.5.5.1. A copy of the ROI shall be placed into each individual case record.

9.5.5.2. It is essential that enrolled Veterans permit communication between VA and Contractor by Release of Information to ensure safe and effective treatment of each Veteran.

9.5.5.2.1. Veterans who refuse to allow this communication about their treatment needs and ongoing care will need to receive services elsewhere.

9.6. ABSENCES AND CANCELLATIONS

9.6.1. Per VHA Directive 1162.04(1), HCHV CRS program allows payment to a Contractor for an absent Veteran under the following conditions:

9.6.1.1. Scheduled Absences

9.6.1.1.1. VA will pay per diem up to a maximum of 96 hours per month for the scheduled absence(s) of a Veteran.

9.6.1.1.2. Contractor shall notify VA Liaison, COR, or designee in writing of all Veteran requests for scheduled absences or “passes” prior to the start of any absence from the program authorized by Contractor, preferably 24-48 hours in advance, and include reason for pass and contact/location information for Veteran while absent. To receive payment, the absence must:

9.6.1.1.2.1. Be pre-planned, consistent with and support the Veteran’s individual service plan (e.g., family reunification, short-term medical, substance use disorder or psychiatric treatment).

9.6.1.1.2.2. Have the reason documented in the individual Veteran’s case file, treatment record or service plan.

9.6.1.1.2.3. Not result in the bed being filled by the provider.

9.6.1.1.2.4. Not be for a break or vacation from treatment.

9.6.1.1.2.5. Not be used for extended educational or employment circumstances.

9.6.1.1.2.6. Not be used to create more than four (4) consecutive days of absence.

9.6.1.2. Unscheduled Absences

9.6.1.2.1. VA will pay per diem up to a maximum of 48 consecutive hours for the unscheduled absence. To receive payment for an unscheduled absence:

- 9.6.1.2.1.1. Contractor must have evidence of active outreach to locate and reengage the Veteran and document the steps taken in the Veterans individual case file, treatment record, or service plan.
- 9.6.1.2.1.2. Contractor shall notify VA Liaison, COR or Designee immediately or during next available business hours of all unscheduled Veteran absences.
- 9.6.1.2.1.3. Contractor shall not fill the bed during this time.
- 9.6.1.2.1.4. A Veteran who is absent without Contractor approval must be discharged from the HCHV CRS program if not located within 48 hours.

9.6.1.3. Inpatient Hospitalization

- 9.6.1.3.1. VA will pay per diem up to a maximum of seven (7) days if a Veteran enrolled in HCHV LDSH requires inpatient medical or psychiatric treatment. To receive payment while a Veteran is hospitalized as an inpatient:
 - 9.6.1.3.1.1. Contractor shall notify VA Safe Haven Liaison, COR, or Designee immediately or during next available business hours of all hospital admissions.
 - 9.6.1.3.1.2. Contractor shall not fill the bed during this time.
 - 9.6.1.3.1.3. Contractor shall maintain contact with the HCHV Safe Haven Liaison and the Veteran if authorized for continued payment.

9.6.1.4. Ineligible Veteran

- 9.6.1.4.1. When a Veteran is admitted to an HCHV LDSH program and found to be ineligible for HCHV LDSH services, VA will pay for a maximum of four (4) days from the date of admission to allow Contractor and HCHV Safe Haven Liaison or Designee time to locate and arrange alternate placement.

- 9.6.1.5. Contractor may choose to maintain enrollment of a Veteran **without payment** beyond the approved absence period at Contractor's discretion **and** in consultation with VA Liaison, COR or Designee. However, the Veteran must be discharged from the HCHV LDSH program under the following conditions:
 - 9.6.1.5.1. If the Veteran has not returned within two additional days of the total absence period approved for payment.
 - 9.6.1.5.2. If another Veteran is seeking placement and the program is otherwise full.

9.6.1.6. Contractor cannot receive payment of per diem if/when an enrolled Veteran is temporarily placed in another VA Homeless Programs-funded bed such as HCHV CRS-contracted detox bed or medical respite program.

9.6.1.7. VA reserves the right to remove any or all Veterans from the program at any time without additional cost when it is determined to be in the best interest of the Veteran or VA.

9.7. EXITS TO PERMANENT HOUSING

9.7.1. Contractor shall promote a focus on achieving stable, independent housing for all Veterans enrolled in the HCHV LDSH program.

9.7.2. Exits to permanent, independent housing will be monitored as an indicator of overall program quality utilizing data provided by the VHA Support Service Center's Homeless Services Scorecard.

9.7.3. The target values for exits to this type of housing are established by VHA Homeless Program Office on an annual basis at the start of each fiscal year (e.g., FY26 target value for Exits to Permanent Housing is 55% or more) and can vary from year to year.

9.7.4. Target values for Exits to Permanent Housing will be communicated by VA Safe Haven Liaison and/or COR to Contractor at the start of each fiscal year.

9.7.5. If it is determined that a Veteran is not appropriate for independent living while enrolled in the program, Contractor will assist with identifying and securing alternative options for housing such as Board and Care, Veterans Homes, Sober Living Environment or linking to appropriate higher level of care.

9.8. NEGATIVE EXITS

9.8.1. Contractor shall facilitate Veteran completion of the HCHV LDSH program to the maximum extent possible while still maintaining program integrity and safety.

9.8.2. Negative Exits are defined as exits from the program in which:

9.8.2.1. A Veteran was asked to leave due to violation of program rules (excluding exits due to threatened/actual violence to self or others),

9.8.2.2. A Veteran was asked to leave due to failure to comply with program requirements, or

9.8.2.3. A Veteran leaves the program without consulting staff.

9.8.3. Negative Exits shall be monitored on a continuous basis utilizing data provided by the VHA Support Service Center's Homeless Services Scorecard.

9.8.4. The target values for Negative Exits are established by VHA Homeless Program Office on an annual basis at the start of each fiscal year (e.g., FY26 target value for Negative Exits is 20% or less) and can vary from year to year.

- 9.8.5.** Target values for Negative Exits will be communicated by VA Safe Haven Liaison and/or COR to Contractor at the start of each fiscal year.

9.9. PROCEDURES FOR NEGATIVE EXITS

- 9.9.1.** For all negative exits other than exits for threatened/actual violence to self or others, Contractor shall develop a written progressive warning policy/procedure that includes issuance of at least two formal warnings prior to proceeding with a negative discharge.
- 9.9.2.** The policy/procedure shall include case conference with Program Staff, Veteran, and VA Safe Haven Liaison or Designee, and/or other relevant clinical providers involved in the Veteran's care when indicated, e.g., HUD-VASH Social Worker or Mental Health/Substance Use Disorder staff, no later than issuance of the final warning.
- 9.9.3.** Information on the policy/procedure shall be provided to the Veteran upon admission to the program including a formal appeals process through which Veteran may appeal program regulations, sanctions or discharges.
- 9.9.4.** There shall be no permanent bans from the program/facility.
- 9.9.5.** Veterans exited under negative circumstances shall be notified upon request of conditions or process for re-admission.
- 9.9.6.** Contractor shall be expected to make reasonable efforts to facilitate referral to another facility or appropriate level of care if negative discharge is indicated.
- 9.9.6.1.** Direct discharge to SFVAHCS Hospitals or Clinics for alternative resources and referrals without consulting HCHV Liaison on day of discharge is not considered an appropriate level of care.
- 9.9.7.** The policy/procedure for negative exits shall be subject to approval by VA Liaison, COR and/or Designee.

9.10. INDIVIDUAL CASE RECORDS

- 9.10.1.** Contractor shall maintain an individual case record for each enrolled Veteran.
- 9.10.2.** Case records shall be maintained in compliance with the requirements of the "Confidentiality of Alcohol and Drug Abuse Patient Records" (42 CFR part II) and the "Confidentiality of Certain Medical Records" (38 U.S.C. 7332), and in accordance with the Health Insurance Portability and Accountability Act (HIPAA: Pub. Law. 104-191).
- 9.10.3.** Case records should contain at a minimum:
- 9.10.3.1.** Reason for referral
 - 9.10.3.2.** Pertinent demographic information
 - 9.10.3.3.** Verification of Veteran status
 - 9.10.3.4.** Sociocultural assessment including:
 - 9.10.3.4.1.** Housing History and Needs Assessment

information when transferring Veteran information via electronic means, such as faxing or system-to-system transmission.

9.11.5. Contractor shall not use personal flash drives to store or retain any personal health information (PHI) or Veteran information.

9.11.6. Contractor shall not transmit any PHI via email without use of encryption software that uses VA-approved authentication mechanisms.

9.11.6.1. While the first initial of the last name of a Veteran and the last four of their Social Security Number by itself is not considered individually identifiable, any other PHI that has not been de-identified in accordance with VHA Directive 1605.04 coupled with the last initial/last four may not be transmitted together without encryption.

9.12. HOMELESS MANAGEMENT INFORMATION SYSTEMS (HMIS) DATA REPORTING

9.12.1. Veterans served through VA's HCHV LDSH program shall have client level data entered into the local Community Continuum of Care's (CoC) HMIS.

9.12.2. Data entered must include, at a minimum, the Universal Data Elements from the 2026 HMIS Data Standards.

9.12.3. Contractor is responsible for registering with local HMIS system to obtain access roles and manage agency data within the system.

9.13. DAILY CENSUS SIGN-IN SHEET

9.13.1. Contractor shall collect Veteran participant signatures on a daily census sign-in sheet to verify attendance in the program.

9.13.2. This information is to be provided to the VA Liaison, COR, or other designee at a frequency to be designated by VA that is no less than once weekly, and up to daily, with weekend or holiday sign-in sheets to be provided on the next business day in order to ensure effective monitoring of bed utilization.

9.13.2.1. INCIDENT REPORTING

9.13.2.2. Contractor shall notify the VA immediately when *any* adverse critical incident involving a Veteran admitted to the program occurs. Critical Incidents shall include, but are not limited to:

9.13.2.2.1. Falls

9.13.2.2.2. Elderly/Dependent Adult Abuse or Neglect

9.13.2.2.3. Sexual Assault

9.13.2.2.4. Fire (Veteran Involved)

9.13.2.2.5. Medical or Mental Health Emergency (911/988) Calls

9.13.2.2.6. Hospitalization

9.13.2.2.7. Suicidal ideation or attempt

9.13.2.2.8. Homicidal ideation

extension exceeding six (6) months from date of admission shall be approved in writing by VA Liaison *and* COR or designee.

9.15. GRIEVANCE PROCEDURES

- 9.15.1.** Contractor shall have an internal grievance process that Veterans can use to resolve conflicts within the program.
 - 9.15.1.1.** Contractor shall have written policies and procedures for resolving grievances including a statement regarding the client's right to request reasonable accommodation.
 - 9.15.1.2.** Contractor shall post the grievance procedure in a conspicuous place that is accessible to enrolled Veterans.
 - 9.15.1.3.** Each Veteran shall receive a copy of the grievance policies and procedures upon intake and upon receiving a warning or notice of discharge.
- 9.15.2.** Contractor shall refer grievances received directly from Veterans to the identified VA Liaison, COR or designee within 24 business hours of complaint.
- 9.15.3.** All grievances received by the VA Liaison, COR or designee will be forwarded to the Contractor within 24 business hours.
- 9.15.4.** Contractor shall conduct an investigation of the grievance and respond to the VA Liaison or COR within five (5) business days or less with proposed resolution or plan for corrective action.
- 9.15.5.** The Contracting Officer (CO) shall be notified in instances where the proposed course of action or response does not appear sufficient to resolve the grievance.
- 9.15.6.** VA Liaison, COR or designee will monitor the services being provided in this contract program.
- 9.15.7.** Contractor shall cooperate and collaborate with VA Staff and COR by providing information and answering questions in a timely manner when requested.
- 9.15.8.** Contractor will be subject to a Contract Discrepancy Report (CDR) anytime unacceptable contract performance is determined to be critical in nature and requires formal corrective action through report of contract discrepancy to CO.
- 9.15.9.** Failure to perform duties herein will have Contractor be subject to contract termination.

9.16. SATISFACTION SURVEYS

- 9.16.1.** When requested, Contractor shall administer, collect, and deliver to VA Liaison or Designee a survey of the Veterans' experience in the program.

9.17. ADMISSION AND DISCHARGE REPORTING

- 9.17.1.** Contractor shall notify the identified VA Liaison, COR, and/or Designee within 24 business hours that an admission or discharge has occurred.
- 9.17.2.** Contractor shall provide a written report of admission or discharge within 48 business hours that contains the specific data required for the Homeless

Operations and Management Evaluation System (HOMES), VA's primary platform for collecting intake, progress, and outcome information for homeless Veterans as they move through VA's system of care.

9.17.3. The written report of discharge shall also contain a brief discharge summary that includes but is not limited to:

9.17.3.1. Date of exit

9.17.3.2. Type of exit (positive vs. negative vs. neutral)

9.17.3.3. Veteran's perception of exit in instances where a Veteran was terminated from the program

9.17.3.4. Status of treatment goals at the time of exit

9.17.3.5. Sentinel events occurring during the episode of care

9.17.3.6. Established aftercare plan

9.17.3.7. Housing status at exit

9.17.3.8. Future contact information

9.18. CONDUCT

9.18.1. Contractor shall provide housing and supportive services in a manner that is free from religious discrimination.

9.18.2. Local law enforcement and/or fire departments should be contacted for assistance and intervention as appropriate and indicated by any given circumstances.

9.18.3. In the event of a medical or psychiatric emergency, it is agreed that every effort will be made to facilitate Veteran access to local VA Medical Center for care.

9.18.3.1. If a VA Medical Center is not available in the vicinity, or is otherwise inconveniently located, Contractor will advise the VA Liaison, COR or AOD of the facility to which the Veteran has been transported and/or admitted.

9.18.3.2. Contractor will also be expected to assist Veterans requiring non-urgent services with accessing appropriate care from a VA or community facility, as appropriate.

9.19. TRAVEL

9.19.1. Contractor shall assist Veterans with arranging local transportation to scheduled meetings and appointments.

9.19.2. Contractor shall help Veterans understand and learn how to use public transportation.

9.19.2.1. This includes providing education, information and clarifying instructions necessary to effectively utilize public transit systems.

9.19.3. If Contractor and/or VA staff determine that public transportation is not available, adequate, or appropriate for any Veteran, Contractor will be expected to assist the Veteran with identifying alternative modes of transport.

9.19.4. Contractor shall be expected to accompany Veteran to appointments to ensure Veteran linkage with provider should any enrolled Veteran need this additional assistance.

9.19.5. Contractor is not required to provide transportation to a Veteran directly.

9.20. FACILITIES

9.20.1. GENERAL REQUIREMENTS: It is the responsibility of Contractor to properly maintain its facilities and the VA shall have no responsibility for paying or reimbursing the Contractor for such expenses. The contract facility shall:

- 9.20.1.1.** Where applicable, have a current occupancy permit issued by the local and state governments in the jurisdiction where the facility is located.
- 9.20.1.2.** Be in compliance with existing standards of State safety codes and local, and/or State health and sanitation codes.
- 9.20.1.3.** Meet the requirements of the Americans with Disabilities Act (ADA) (Public Law 100-336, 42 USC 12101-12213) pertaining to handicapped accessibility in effect on the date of contract award.
- 9.20.1.4.** Where applicable, be licensed under State or local authority.
- 9.20.1.5.** Where applicable, be accredited by the State.
- 9.20.1.6.** Be equipped with operational air conditioning/heating systems if needed to maintain acceptable temperature.
- 9.20.1.7.** Be kept clean, free of dirt, grime, mold, or other hazardous substances and damage that noticeably detracts from the overall appearance.
- 9.20.1.8.** Be equipped with first aid equipment and an evacuation plan in case of emergency.
- 9.20.1.9.** Have windows and doors that can be opened and closed in accordance with manufacturer's standards.
- 9.20.1.10.** Have an aggressive ongoing plan to address bed bug infestation.
 - 9.20.1.10.1.** Ongoing bed bug infestation will be grounds for immediate discharge of Veterans from the facility.
- 9.20.1.11.** Have emergency and disaster plans with written protocols that are posted to guide staff response to crises including, but not limited to, manmade and natural disasters, episodes of infectious diseases, physical injury, program participant suicide or suicide attempts, overdoses, and domestic or other violence.
 - 9.20.1.11.1.** All Contractor staff shall be trained on emergency procedures and protocols.

9.21. FIRE SAFETY REQUIREMENTS

- 9.21.1.** The building must meet the requirements in the applicable provisions of NFPA 101 and the other publications referenced in those provisions. Any equivalencies or variances to VA requirements must be approved by the appropriate Veterans Health Administration Veterans Integrated Service Network (VISN) Director or Designee.
 - 9.21.1.1.** A written fire plan for evacuation in the event of fire shall be developed and reviewed annually.
 - 9.21.1.1.1.** The plan shall outline the duties, responsibilities, and

actions to be taken by the staff and residents in the event of a fire emergency.

9.21.1.1.2. This plan shall be implemented during fire drills.

9.21.2. Fire drills shall be conducted at least quarterly (monthly preferred due to frequency in change in program participants and in staffing across 24/7 staffed shifts).

9.21.2.1. Program participants shall be instructed in evacuation procedures when the primary and/or secondary exits are blocked.

9.21.2.2. A written policy regarding tobacco smoking in the facility shall be established and enforced.

9.21.3. Portable fire extinguishers shall be installed at the facility.

9.21.3.1. Contractor shall use NFPA 101, Portable Fire Extinguishers, as guidance in selection and location requirements of extinguishers.

9.21.3.2. Requirements for fire protection equipment and systems shall be in accordance with NFPA 101.

9.21.3.3. All fire protection systems and equipment, such as the fire alarm system, smoke detectors, and portable extinguishers, shall be inspected, tested and maintained in accordance with the applicable NFPA fire codes and the results documented.

9.22. INSPECTIONS

9.22.1. Prior to Veteran placement under this contract and annually thereafter during any subsequent contracted performance periods, a multidisciplinary VA team consisting of a social worker, dietitian or nutrition and food service professional, nursing staff, VA Police, and a Safety and Occupational Health Specialist, as well as any other subject matter experts determined necessary by the medical center director, COR, HCHV Coordinator, or VA Liaison, shall conduct a survey of the Contractor's facilities to be used to provide Veterans food, shelter, and clinical services to assure the facility provides acceptable level quality care in a safe environment.

9.22.2. Additional inspections may also be carried out, announced or unannounced, at any other time as deemed necessary by VA.

9.22.3. Contractor shall be advised of the findings of the annual inspection team.

9.22.3.1. If deficiencies are noted during any inspection, Contractor will be given a reasonable amount of time (typically 30 days) to take corrective action and to notify the VA Liaison, Contracting Officer's Representative, and/or other VA Designee that the corrections have been made.

9.22.3.2. A contract will not be approved for Veteran placement until noted deficiencies have been eliminated.

9.22.3.3. Failure by Contractor to take corrective action within the reasonable time provided will be reported to the VA Contracting Officer.

9.22.3.4. If corrections are not made to the satisfaction of the VA, the Contracting Officer will be notified and shall be the final arbiter on the necessary resulting consequences and action.

9.22.4. Annual reinspection shall be unannounced whenever possible.

9.22.4.1. When an unannounced annual inspection is not possible due to privacy or other concerns, no less than four unannounced site visits by HCHV Liaison or other VA Designee shall be made annually in addition to annual reinspection and shall include:

9.22.4.1.1. A visual safety and sanitation inspection of the facility including meal preparation areas, fire exits, sleeping areas and medication storage.

9.22.4.1.1.1. Any safety or sanitation deficiency shall be addressed immediately.

9.22.4.1.2. Review of Contractor's emergency and disaster plans to ensure they are up to date and that staff are trained in the procedures outlined in the plans.

9.22.4.1.3. An audit of Veteran's clinical records selected randomly to ensure documentation of case management services and that other services are being provided as required by the contract terms.

9.22.4.1.4. Review of any Veteran complaints to ensure that these have been resolved in a fair, impartial and consistent manner.

9.22.4.2. The results of the unannounced site visits are to be documented and shared with the COR and the Contracting Officer.

9.22.4.3. The report and any issues that were found along with the resolution must be kept in the COR Administrative File for that program.

9.22.5. The inspection of the Contractor facilities shall include inspection for conformity to the current Life Safety Code as described in Section 8.22. and shall also include the following:

9.22.5.1. General observation of enrolled Veterans to determine if they maintain an acceptable level of personal hygiene and grooming.

9.22.5.2. Assessment of whether the facility meets applicable fire, safety and sanitation standards.

9.22.5.3. Determining whether the facility is in attractive surroundings conducive to social interaction and the fullest development of the enrolled Veterans' rehabilitative potential.

9.22.5.4. Observation of facility operations to see if appropriate organized activity programs are available during waking hours (including evenings) and the degree to which a high level of activity is observed in the facility, such as individual professional counseling, physical activities, assistance with health and personal hygiene.

9.22.5.5. Seeking evidence of facility-community interaction, demonstrated by the nature of scheduled activities or by information about resident flow out of the facility, e.g., community activities, volunteers, local consumer services.

9.22.5.6. Observation of staff behavior and interaction with residents to

determine if they convey an attitude of genuine concern and caring.

9.22.5.7. Inspecting the types of meals and other nutrition provided to residents to see if appetizing, nutritionally adequate meals are provided in a setting, which encourages social interaction and if nutritious snacks between meals and bedtime are available for those requiring or desiring additional food, when it is not medically contraindicated.

9.22.5.8. Making a spot check of Veterans' records to ensure accuracy with respect to Veterans' length of stay and services provided to the Veterans.

9.22.6. All Department of Veterans Affairs inspection findings for residential facilities furnishing treatment and rehabilitative services to eligible Veterans shall, to the extent necessary, be made available to all government agencies charged with the responsibility of licensing or otherwise regulating or inspecting such institutions.

10. DELIVERABLES

10.1. QUALITY CONTROL BUSINESS PLAN

10.1.1. Contractor shall submit a Quality Control Business Plan that supports the program objectives and associated tasks. The Contracting Officer shall review and comment as necessary to ensure that contract goals are met.

10.2. STAFFING AND SERVICE PLAN

10.2.1. Contractor shall provide a detailed staffing and service plan. Plan shall demonstrate that sufficient professional personnel are employed to carry out the policies, responsibilities, and services required under this contract.

10.2.2. Contractor shall identify each person functioning as "Key Personnel" under this contract and provide VA with a description of the services to be provided by each person.

10.2.3. Contractor shall assign to this contract personnel who are qualified by education, training, and, when required, certification or licensure, to provide the Therapeutic, Rehabilitative and other basic services required by this Performance Work Statement.

10.2.4. Contractor shall be required to supply a resume(s) summarizing relevant skills and experience of any/all Key Personnel. Minimum Key Personnel requirements are as follows:

10.2.4.1. One full-time **Program Manager** who shall be responsible for contract performance, who shall have authority to act for Contractor on all matters relating to daily operations of the program, and who shall be available for consultation with the authority to make decisions regarding the facility and residents in case of emergency 24 hours a day, 7 days a week.

10.2.4.2. Program Manager shall have the following qualifications:

10.2.4.2.1. Professionally credentialed clinical supervisor, i.e., LCSW, PhD/PsyD, LPCC, LMFT, ***strongly preferred***, or;

10.2.4.2.2. Master's degree in Social Work, Psychology, Counseling or closely related field of concentration may be accepted with the following:

10.2.4.2.2.1. Registration with CA Board of Behavioral Sciences (BBS) required; proof of application for registration required within 30 days of employment.

10.2.4.2.2.2. Licensure hours for LCSW, MFT, LPCC, or Psychology shall be provided by Contractor.

10.2.4.2.2.3. Unlicensed Master's Level Program Manager shall obtain professional licensure within five years of active registration with BBS.

10.2.4.2.2.4. At least three (3) years' experience in a residential services treatment setting including supervisory experience or management responsibilities of clinical staff, or;

10.2.4.2.3. Bachelor's degree in Social Work, Psychology, Community Mental Health or closely related field of concentration may be considered in place of a Master's degree if:

10.2.4.2.3.1. At least six (6) years' experience in residential treatment, community mental health, SUD services, and/or social rehabilitation ***including*** supervisory experience.

10.2.4.3. If Program Manager is unlicensed, then one professionally credentialed clinician (i.e., LCSW, PhD, PsyD, LMFT, LPCC) within the contract program's direct chain of command shall be available to make regular onsite visits and provide ongoing clinical supervision of cases. This clinician may also serve in the role of case management/counseling personnel but is not required to do so.

10.2.4.4. Sufficient case management/counseling personnel to provide clinical case management, ongoing care planning and housing placement services, to maintain the therapeutic milieu and to facilitate prosocial activities for enrolled Veterans.

10.2.4.4.1. VA recommends a staffing ratio of one full-time dedicated intensive case manager per 10 Veterans enrolled in the LDSH program. Consideration will be given by VA to the unique staffing characteristics of the contract facility. Exceptions may be granted in writing by VA.

10.2.4.4.2. Each Veteran shall have an assigned case manager responsible for coordinating and providing the care and supportive services as specified in the contract.

- 10.2.4.4.3.** Case managers shall be available to provide direct services, at a minimum, during regular business hours, Monday-Friday 8AM-4:30PM.
- 10.2.4.4.4.** Case managers shall have sufficient training and experience working with individuals experiencing homelessness.
- 10.2.4.4.5.** Case managers shall have knowledge of the Recovery Model, trauma-informed and strengths-based care, and the stages of change.
- 10.2.4.4.6.** Case managers shall be able to demonstrate flexibility and adaptability, a willingness to meet enrolled Veterans where they are, leave rules of sobriety-based programming behind, and tolerate minor infractions of rules while using them as opportunities for engagement.
- 10.2.4.4.7.** Experience working with Veterans and individuals dealing with chronic medical, mental health and substance misuse is highly desirable.
- 10.2.4.4.8.** Master's level clinical training preferred
- 10.2.4.4.9.** Bachelor's level social service training and/or equivalent experience accepted.
- 10.2.4.5.** At least one staff member is onsite and on duty 24 hours a day including evenings, nights, weekends and holidays for supervision and safety of the LDSH program community.
 - 10.2.4.5.1.** Contact information for the Program Manager or other Clinical Designee shall be available to the staff member(s) on duty.
 - 10.2.4.5.2.** Program Manager or other Clinical Designee shall be capable of responding within 30 minutes in case of emergency 24 hours a day, 7 days a week.
 - 10.2.4.5.3.** **Security staff alone, whether employed directly by Contractor or subcontracted by Contractor, is not sufficient to meet this requirement.**
- 10.2.4.6.** Other personnel necessary for safe operation of services contracted by VA such as janitorial, security, kitchen, front desk, maintenance, and general milieu staff shall be provided by Contractor which are compatible with maintaining quality care programming and the safety of the Veterans, other program participants, and staff.
- 10.2.4.7.** Contractor shall provide resumes for any proposed substitutions of Key Personnel at least 15 days prior to projected date of substitution. The Contracting Officer (CO) shall notify Contractor within fifteen (15) calendar days after receipt of all required information if the VA is able to accept the proposed substitute key personnel.
 - 10.2.4.7.1.** Temporary substitutions of key personnel shall be permitted

in accordance with Contractor's contingency plan. Contractor's contingency plan to be utilized if personnel leave Contractor's employment or are unable to continue performance in accordance with the terms and conditions of the resulting contract shall be submitted to the CO as a part of the proposal package.

10.2.4.7.2. Contingency plan shall include Contractor's immediate notification of VA Liaison, COR and/or Designee of cross coverage by alternative staff if a primary Contractor staff is temporarily absent. The alternate staff shall be made aware of the HCHV LDSH contract requirements.

10.2.4.7.3. The CO is the ultimate authority on acceptable length for temporary substitution of Key Personnel.

10.2.4.8. The VA reserves the right to refuse or revoke acceptance of any key personnel if personal or professional conduct, or lack of required skills or experience jeopardizes patient care or interferes with the regular and ordinary operation of the facility and the HCHV LDSH program.

10.2.4.9. At least one staff during each shift shall have CPR or BLS certification and be available in an emergency, 24 hours per day, 7 days per week.

10.2.4.10. *All staff* working with enrolled Veterans in the HCHV LDSH program shall be trained in:

10.2.4.10.1. Motivational Interviewing

10.2.4.10.2. Harm Reduction

10.2.4.10.3. Crisis Management and Intervention

10.2.4.10.4. De-Escalation Techniques

10.2.4.11. It is preferable that Contractor make training and education available to all staff at least once per year in the following areas:

10.2.4.11.1. CPR/First Aid/Universal Precautions (disease transmission prevention)

10.2.4.11.2. Naloxone/Overdose Prevention

10.2.4.11.3. Suicide Prevention

10.2.4.11.4. Conflict Resolution

10.2.4.11.5. Mandated Reporting Laws

10.2.4.11.6. Professional Boundaries

10.2.4.11.7. Cultural Humility

10.2.4.11.8. Sexual Harassment

10.2.4.11.9. Permanent Housing Attainment

10.2.4.11.10. Sensitivity to Wider Issues of Homelessness

10.2.4.11.11. Clinical topics such as Stages of Change, Severe Mental Illness, Countertransference, Low Barrier/Low Demand Interventions

10.3. REFERRAL REPORTS

10.3.1. Contractor shall provide a written report to VA Liaison, COR, and/or Designee of all referrals made to the program at a frequency designated by VA that is no less than once monthly and shall include:

- 10.3.1.1.** Name/Last Four of Referred Veteran
- 10.3.1.2.** Date/time of referral receipt
- 10.3.1.3.** Date/time of admissions decision
- 10.3.1.4.** Date of admission, or;
- 10.3.1.5.** Reason for denial or for otherwise not being admitted
- 10.3.1.6.** Reason for delay in admission, if indicated

10.4. DOCUMENTATION OF SUPPORTIVE SERVICES

10.4.1. Contractor shall provide written documentation constituted by the individual Veteran case record that verifies the provision of all supportive services required under this contract for each enrolled Veteran.

10.5. CARE PLAN

10.5.1. The written plan of care shall be completed and entered into the individual Veteran case record no later than day 14 days after being admitted to the program. Care plans will be reviewed and updated no less than once every 30 days.

10.6. CRITICAL INCIDENT REPORTS

10.6.1. Written critical incident reports shall be submitted to the VA Liaison, COR, and/or Designee within 24 business hours (VA Liaison/COR to be notified immediately or as soon as possible at start of the following business day when incidents occur; AOD to be contacted if immediate VA intervention/support is needed after hours).

10.7. EXTENSION REQUESTS

10.7.1. Written requests for extension are due prior to the 180th day when it is anticipated that the enrolled Veteran will require additional time beyond the initially authorized service period, and *prior* to the expiration of the initial and any and all subsequent future extension authorizations.

10.8. ADMISSION AND DISCHARGE REPORTS

10.8.1. Entry and exit notification forms and brief discharge summary shall be completed and submitted to the VA Liaison and/or COR or designee within 48 business hours of admission or discharge (VA Liaison to be notified verbally or via secure email immediately or within 24 business hours of admission or discharge).

10.9. SATISFACTION SURVEYS

10.9.1. When Veteran participant surveys/questionnaires (e.g., CHALENG Survey) are provided by VA to the Contractor to give to Veterans, completed surveys are to be

returned to VA Liaison at the end of each designated survey period.

10.10. DAILY CENSUS SIGN-IN SHEET

10.10.1. The daily census sign-in sheet shall be provided to the VA Liaison, COR and/or designee at a frequency designated by VA that is no less than once weekly and up to daily with weekend or holiday sign-in sheets to be provided on the next business day.

10.11. NOTIFICATION OF ABSENCES

10.11.1. Contractor shall notify VA Liaison, COR and/or Designee immediately or during next available business hours of all unauthorized Veteran absences from the Contractor facility.

10.11.2. Contractor shall notify VA Liaison, COR, or Designee in writing of all Veteran requests for passes prior to start, preferably 24-48 hours, of any absence from program authorized by Contractor.

10.12. INVOICES

10.12.1. Unless specifically excluded in this contract, the per diem rate established will include all services listed in this document and all services or supplies normally provided to other residents of the facility at no extra charge.

10.12.2. Payments made by VA under this contract shall constitute total cost of care and housing for the homeless Veterans enrolled in this program.

10.12.3. The monthly invoice is computed at the per diem rate multiplied by the total number of beds occupied by Veterans at midnight each night of the given month and does not include day of discharge.

10.12.4. Invoices shall first be submitted to the VA Liaison, COR, or designee for approval by the 5th of the month immediately following the billing period in question.

10.12.5. Once approved, invoices are to be submitted through the Tungsten Network (Electronic Invoicing System) by the 10th of the month immediately following the billing period in question.

10.12.6. All electronic invoices submitted shall be accompanied by the invoice pre-approved by VA Liaison, COR, or Designee for reference of the certifying official. (For additional information, Reference: VAAR 852.273-72 Electronic Submission of Payments pg 23. and FAR 52.232-33 Payments by Electronic Funds Transfer—System for Award Management pg.27.).

10.13. QUARTERLY PERFORMANCE REPORT

10.13.1. Upon request, the Contractor shall provide the COR with a written report detailing program data and activities on a quarterly basis. The report should contain, at minimum, the following information:

10.13.1.1. Total Number of Veterans Served

10.13.1.2. Occupancy Rate

10.13.1.3. Percentage of Veterans discharged to permanent, independent

housing

10.13.1.4. Percentage of Negative Exits i.e. discharges due to rule violation, failure to comply with program requirements, or unexpected discharges without prior consultation with staff.

10.13.1.5. Other information the Contractor feels pertinent, such as quality improvement projects, changes in staffing or business practices, systems or resource concerns.

10.14. GRIEVANCES

10.14.1. Contractor shall refer grievances received directly from Veterans to the identified VA Liaison, COR or Designee within 24 hours of complaint.

10.14.2. Contractor shall conduct an investigation of the grievance and respond to the VA Liaison or COR within five (5) business days or less with proposed resolution or plan for corrective action.

10.15. CONTRACT DISCREPANCY REPORTS (CDR)

10.15.1. Contractor will be subject to a CDR anytime unacceptable contract performance is determined to be critical in nature and requires formal corrective action involving reporting the discrepancy to CO by CDR.

10.15.2. Depending on the nature of the violation, Contractor will be required to submit a corrective action plan to COR or Designee within a timeframe designated by COR or Designee based on the severity of the violation, e.g., safety issues.

10.15.3. The program will be monitored for performance.

10.15.4. Failure to perform duties herein will have Contractor be subject to contract termination.

10.16. INSPECTIONS

10.16.1. Prior to placement of Veterans in the Contract facility and annually thereafter, during any subsequent contracted performance periods, a multidisciplinary VA team shall conduct a survey of Contractor's facilities to be used to provide Veterans' food, shelter, and clinical services to ensure the facility provides acceptable level quality care in a safe environment. Additional inspections may also be carried out, announced or unannounced, at any other time as deemed necessary by VA.

DELIVERABLE TIMETABLE

Deliverables	Due Dates
10.1. QUALITY CONTROL BUSINESS PLAN	Due upon solicitation close date
10.2. STAFFING AND SERVICE PLAN	Due upon solicitation close date

10.3. REFERRAL REPORTS	Due to VA Liaison/COR/Designee at frequency designated by VA at least once monthly
10.4. DOCUMENTATION OF SUPPORTIVE SERVICES	Due upon request with minimum of quarterly reviews to be completed by VA
10.5. CARE PLAN	Due in Veteran case record by day 14; due to VA upon request
10.6. CRITICAL INCIDENT REPORTS	Due within 24 business hours of a critical adverse event involving a Veteran
10.7. EXTENSION REQUESTS	Due prior to the expiration of any authorized period of service
10.8. ADMISSION AND DISCHARGE REPORTS	Due within 48 business hours of Veteran admission or discharge
10.9. SATISFACTION SURVEYS	Due at the end of each monthly service period when requested
10.10. DAILY CENSUS SIGN-IN SHEET	Due to VA Liaison/COR/designee at frequency designated by VA at least weekly and up to daily
10.11. NOTIFICATION OF ABSENCES	Due to VA Liaison/COR/designee immediately or during next available business hours if absence is unauthorized; if authorized, then due prior to start of absence, preferably 24-48 hours
10.12. INVOICES	Due to VA Liaison, COR or designee by the 5 th of the month immediately following the billing period in question; due in Tungsten Network (electronic billing system) by the 10 th of the month immediately following the billing period in question.
10.13. QUARTERLY PERFORMANCE REPORT	Due to VA Liaison/COR/designee by the 15 th of the month following the end of each FY quarter upon request
10.14. GRIEVANCES	Due to VA Liaison/COR/designee within 48 hours of Veteran complaint; proposed resolution/plan of corrective action due within five business days of investigation

10.15. CONTRACT DISCREPANCY REPORTS	Due to COR/designee within time-frame designated by COR based on severity of violation
10.16. INSPECTIONS	Due prior to Veteran placement under the contract terms and annually thereafter or at any time deemed necessary by VA

11. CERTIFICATION & ACCREDITATION REQUIREMENTS

11.1. The C&A requirements do not apply, and a Security Accreditation Package is not required.

12. CONTRACT CHANGES/TECHNICAL DIRECTIONS

12.1. The Contracting Officer is the only person authorized to approve changes or modify any of the requirements of this contract. Contractor shall communicate with the Contracting Officer on all matters pertaining to contract administration. Only the Contracting Officer is authorized to make commitments or issue changes that shall affect price, quantity or quality of performance of this contract.

12.2. IN THE EVENT CONTRACTOR AFFECTS ANY SUCH CHANGE AT THE DIRECTION OF ANY PERSON OTHER THAN THE CONTRACTING OFFICER WITHOUT AUTHORITY, NO ADJUSTMENT SHALL BE MADE IN THE CONTRACT PRICE TO COVER AN INCREASE IN COSTS INCURRED AS A RESULT THEREOF.

12.3. The COR will be responsible for the overall technical administration of this contract as outlined in the COR Delegation of Authority including monitoring of Contractor's performance.

13. VA HANDBOOK 6500.6 CONTRACT SECURITY - APPENDIX C: VA INFORMATION AND INFORMATION SYSTEM SECURITY/PRIVACY LANGUAGE FOR INCLUSION INTO CONTRACTS (VA Notice 24-12; April 22, 2024)

13.1. GENERAL. This entire section applies to all acquisitions requiring any Information Security and Privacy language. Contractors, contractor personnel, subcontractors and subcontractor personnel will be subject to the same federal laws, regulations, standards, VA directives and handbooks, as VA personnel regarding information and information system security and privacy.

13.2. VA INFORMATION CUSTODIAL LANGUAGE. This entire section applies to all acquisitions requiring any Information Security and Privacy language.

13.2.1. The Government shall receive unlimited rights to data/intellectual property first produced and delivered in the performance of this contract or order (hereinafter

“contract”) unless expressly stated otherwise in this contract. This includes all rights to source code and all documentation created in support thereof. The primary clause used to define Government and Contractor data rights is FAR 52.227-14 *Rights in Data – General*. The primary clause used to define computer software license (not data/intellectual property first produced under this contractor or order) is FAR 52.227-19, *Commercial Computer Software License*.

- 13.2.2.** Information made available to the contractor by VA for the performance or administration of this contract will be used only for the purposes specified in the service agreement, SOW, PWS, PD, and/or contract. The contractor shall not use VA information in any other manner without prior written approval from a VA Contracting Officer (CO). The primary clause used to define Government and Contractor data rights is FAR 52.227-14 *Rights in Data – General*.
- 13.2.3.** VA information will not be co-mingled with any other data on the contractor’s information systems or media storage systems. The contractor shall ensure compliance with Federal and VA requirements related to data protection, data encryption, physical data segregation, logical data segregation, classification requirements and media sanitization.
- 13.2.4.** VA reserves the right to conduct scheduled or unscheduled audits, assessments, or investigations of contractor Information Technology (IT) resources to ensure information security is compliant with Federal and VA requirements. The Contractor shall provide all necessary access to records (including electronic and documentary materials related to the contracts and subcontracts) and support (including access to Contractor and Subcontractor staff associated with the contract) to VA, VA's Office Inspector General (OIG), and/or Government Accountability Office (GAO) staff during periodic control assessments, audits, or investigations.
- 13.2.5.** The Contractor may only use VA information within the terms of the contract and applicable Federal law, regulations, and VA policies. If new Federal information security laws, regulations or VA policies become applicable after execution of the contract, the parties agree to negotiate contract modification and adjustment necessary to implement the new laws, regulations, and/or policies.
- 13.2.6.** The Contractor shall not make copies of VA information except as specifically authorized and necessary to perform the terms of the contract. If copies are made for restoration purposes, after the restoration is complete, the copies shall be destroyed in accordance with VA Directive 6500, VA Cybersecurity Program and VA Information Security Knowledge Service.
- 13.2.7.** If a Veterans Health Administration (VHA) contract is terminated for default or cause with a business associate, the related local Business Associate Agreement

(BAA) shall also be terminated and actions taken in accordance with VHA Directive 1605.05, Business Associate Agreements. If there is an executed national BAA associated with the contract, VA will determine what actions are appropriate and notify the contractor.

13.2.8. The Contractor shall store and transmit VA sensitive information in an encrypted form, using VA-approved encryption tools which are, at a minimum, Federal Information Processing Standards (FIPS) 140-2, Security Requirements for Cryptographic Modules (or its successor) validated and in conformance with VA Information Security Knowledge Service requirements. The Contractor shall transmit VA sensitive information using VA approved Transport Layer Security (TLS) configured with FIPS based cipher suites in conformance with National Institute of Standards and Technology (NIST) 800-52, Guidelines for the Selection, Configuration and Use of Transport Layer Security (TLS) Implementations.

13.2.9. The Contractor's firewall and web services security controls, as applicable, shall meet or exceed VA's minimum requirements.

13.2.10. Except for uses and disclosures of VA information authorized by this contract for performance of the contract, the Contractor may use and disclose VA information only in two situations: (i) in response to a qualifying order of a court of competent jurisdiction after notification to VA CO (ii) with written approval from the VA CO. The Contractor shall refer all requests for, demands for production of or inquiries about, VA information and information systems to the VA CO for response.

13.2.11. Notwithstanding the provision above, the Contractor shall not release VA records protected by Title 38 U.S.C. § 5705, Confidentiality of medical quality-assurance records and/or Title 38 U.S.C. § 7332, Confidentiality of certain medical records pertaining to drug addiction, sickle cell anemia, alcoholism or alcohol abuse or infection with Human Immunodeficiency Virus (HIV). If the contractor is in receipt of a court order or other requests for the above-mentioned information, the Contractor shall immediately refer such court order or other requests to the VA CO for response.

13.2.12. Information made available to the Contractor by VA for the performance or administration of this contract or information developed by the contractor in performance or administration of the contract will be protected and secured in accordance with VA Directive 6500 and Identity and Access Management (IAM) Security processes specified in the VA Information Security Knowledge Service.

13.2.13. Any data destruction done on behalf of VA by a Contractor shall be done in accordance with National Archives and Records Administration (NARA) requirements as outlined in VA Directive 6300, Records and Information Management, VA

Handbook 6300.1, Records Management Procedures, and applicable VA Records Control Schedules.

13.2.14. The Contractor shall provide its plan for destruction of all VA data in its possession according to VA Directive 6500 and NIST 800-88, Guidelines for Media Sanitization prior to termination or completion of this contract. If directed by the COR/CO, the contractor shall return all Federal Records to VA for disposition.

13.2.15. Any media, such as paper, magnetic tape, magnetic disks, solid state devices or optical discs that is used to store, process, or access VA information that cannot be destroyed shall be returned to VA. The Contractor shall hold the appropriate material until otherwise directed by the Contracting Officer's Representative (COR) or CO. Items shall be returned securely via VA-approved methods. VA sensitive information must be transmitted utilizing VA-approved encryption tools which are validated under FIPS 140-2 (or its successor) and NIST 800-52. If mailed, the Contractor shall send via a trackable method (USPS, UPS, FedEx, etc.) and immediately provide the COR/CO with the tracking information. Self-certification by the Contractor that the data destruction requirements above have been met shall be sent to the COR/CO within 30 business days of termination of the contract.

13.2.16. All electronic storage media (hard drives, optical disks, CDs, back-up tapes, etc.) used to store, process or access VA information will not be returned to the Contractor at the end of lease, loan, or trade-in. Exceptions to this paragraph will only be granted with the written approval of VA CO.

14. ACCESS TO VA INFORMATION AND VA INFORMATION SYSTEMS. This section applies when any person requires access to information made available to the Contractor by VA for the performance or administration of this contract or information developed by the Contractor in performance or administration of the contract.

14.1. A Contractor/Subcontractor shall request logical (technical) or physical access to VA information and VA information systems for their employees and subcontractors only to the extent necessary to perform the services specified in the solicitation or contract. This includes indirect entities, both affiliate of Contractor/Subcontractor and agent of Contractor/Subcontractor.

14.2. Contractors and Subcontractors shall sign the VA Information Security Rule of Behavior (ROB) before access is provided to VA information and information systems (see Section 4, Training, below). The ROB contains the minimum user compliance requirements and does not supersede any policies of VA facilities or other agency components which provide higher levels of protection to VA's information or information systems. Users who require privileged access shall complete the VA elevated privilege access request processes before privileged access is granted.

- 14.3.** All Contractors and Subcontractors working with VA information are subject to the same security investigative and clearance requirements as those of VA appointees or employees who have access to the same types of information. The level and process of background security investigations for contractors shall be in accordance with VA Directive and Handbook 0710, Personnel Suitability and Security Program. The Office of Human Resources and Administration/Operations, Security and Preparedness (HRA/OSP) is responsible for these policies and procedures. Contract personnel who require access to classified information or information systems shall have an appropriate security clearance. Verification of a Security Clearance shall be processed through the Special Security Officer located in HRA/OSP. Contractors shall conform to all requirements stated in the National Industrial Security Program Operating Manual (NISPOM).
- 14.4.** All Contractors and Subcontractors shall comply with conditions specified in VAAR 852.204-71(d); Contractor operations required to be in United States. All Contractors and subcontractors working with VA information shall be permanently located within a jurisdiction subject to the law of the United States or its Territories to the maximum extent feasible. If services are proposed to be performed abroad the contractor shall state where all non-U.S. services are provided. The Contractor shall deliver to VA a detailed plan specifically addressing communications, personnel control, data protection and potential legal issues. The plan shall be approved by the COR/CO in writing prior to access being granted.
- 14.5.** The Contractor shall notify the COR/CO in writing immediately (no later than 24 hours) after personnel separation or occurrence of other causes. Causes may include the following:
- 14.5.1.** Contractor/Subcontractor personnel no longer has a need for access to VA information or VA information systems. Contractor/Subcontractor personnel are terminated, suspended, or otherwise has their work on a VA project discontinued for any reason.
 - 14.5.2.** Contractor believes their own personnel or Subcontractor personnel may pose a threat to their company's working environment or to any company-owned property. This includes Contractor-owned assets, buildings, confidential data, customers, employees, networks, systems, trade secrets and/or VA data.
 - 14.5.3.** Any previously undisclosed changes to Contractor/Subcontractor background history are brought to light, including but not limited to changes to background investigation or employee record.
 - 14.5.4.** Contractor/Subcontractor personnel have their authorization to work in the United States revoked.
 - 14.5.5.** Agreement by which Contractor provides products and services to VA has either been fulfilled or terminated, such that VA can cut off electronic and/or physical access for Contractor personnel.
- 14.6.** In such cases of contract fulfillment, termination, or other causes; the Contractor shall take the necessary measures to immediately revoke access to VA network, property, information, and information systems (logical and physical) by contractor/subcontractor personnel. These measures include (but are not limited to):

removing and then securing Personal Identity Verification (PIV) badges and PIV – Interoperable (PIV-I) access badges, VA-issued photo badges, credentials for VA facilities and devices, VA-issued laptops, and authentication tokens. Contractors shall notify the appropriate VA COR/CO immediately to initiate access removal.

- 14.7.** Contractors/Subcontractors who no longer require VA accesses will return VA-issued property to VA. This property includes (but is not limited to): documents, electronic equipment, keys, and parking passes. PIV and PIV-I access badges shall be returned to the nearest VA PIV Badge Issuance Office. Once they have had access to VA information, information systems, networks and VA property in their possessions removed, contractors shall notify the appropriate VA COR/CO.

15. TRAINING. This entire section applies to all acquisitions which include **section 3.**

- 15.1.** All Contractors and Subcontractors requiring access to VA information and VA information systems shall successfully complete the following before being granted access to VA information and its systems:

15.1.1. VA Privacy and Information Security Awareness and Rules of Behavior course (Talent Management System (TMS) #10176) initially and annually thereafter.

15.1.2. Sign and acknowledge (electronically through TMS #10176) understanding of and responsibilities for compliance with the Organizational Rules of Behavior, relating to access to VA information and information systems initially and annually thereafter; and

15.1.3. Successfully complete any additional cyber security or privacy training, as required for VA personnel with equivalent information system or information access [to be defined by the VA program official and provided to the VA CO for inclusion in the solicitation document – i.e., any role- based information security training].

- 15.2.** The Contractor shall provide to the COR/CO a copy of the training certificates and certification of signing the Organizational Rules of Behavior for each applicable employee within five days of the initiation of the contract and annually thereafter, as required.

- 15.3.** Failure to complete the mandatory annual training is grounds for suspension or termination of all physical or electronic access privileges and removal from work on the contract until such time as the required training is complete.

16. SECURITY INCIDENT INVESTIGATION. This entire section applies to all acquisitions requiring any Information Security and Privacy language.

- 16.1.** The Contractor, Subcontractor, their employees, or business associates shall immediately (within one hour) report suspected security / privacy incidents to the VA OIT's Enterprise Service Desk (ESD) by calling (855) 673-4357 (TTY: 711). The ESD is OIT's 24/7/365 single point of contact for IT-related issues. After reporting to the ESD, the Contractor, Subcontractor, their employees, or business associates shall, within one hour, provide the COR/CO the incident number received from the ESD.

- 16.2.** To the extent known by the Contractor/Subcontractor, the Contractor/Subcontractor's notice to VA shall identify the information involved and the circumstances surrounding the incident, including the following:

- 16.2.1.** The date and time (or approximation of) the Security Incident occurred.
- 16.2.2.** The names of individuals involved (when applicable).
- 16.2.3.** The physical and logical (if applicable) location of the incident.
- 16.2.4.** Why the Security Incident took place (i.e., catalyst for the failure).
- 16.2.5.** The amount of data belonging to VA believed to have been compromised.
- 16.2.6.** The remediation measures the contractor is taking to ensure no future incidents of a similar nature.
- 16.3.** After the contractor has provided the initial detailed incident summary to VA, they will continue to provide written updates on any new and relevant circumstances or facts they discover. The contractor, subcontractor, and their employees shall fully cooperate with VA or third-party entity performing an independent risk analysis on behalf of VA. Failure to cooperate may be deemed a material breach and grounds for contract termination.
- 16.4.** VA IT Contractors shall follow VA Handbook 6500, Risk Management Framework for VA Information Systems VA Information Security Program, and VA Information Security Knowledge Service guidance for implementing an Incident Response Plan or integrating with an existing VA implementation.
- 16.5.** In instances of theft or break-in or other criminal activity, the Contractor/Subcontractor shall concurrently report the incident to the appropriate law enforcement entity (or entities) of jurisdiction, including the VA OIG, and the VA Office of Security and Law Enforcement. The Contractor, its employees, and its Subcontractors and their employees shall cooperate with VA and any law enforcement authority responsible for the investigation and prosecution of any possible criminal law violation(s) associated with any incident. The contractor/subcontractor shall cooperate with VA in any civil litigation to recover VA information, obtain monetary or other compensation from a third party for damages arising from any incident, or obtain injunctive relief against any third party arising from, or related to, the incident.
- 16.6.** The Contractor shall comply with VA Handbook 6500.2, *Management of Breaches Involving Sensitive Personal Information*, which establishes the breach management policies and assigns responsibilities for the oversight, management and reporting procedures associated with managing of breaches.
- 16.7.** With respect to unsecured Protected Health Information (PHI), the contractor is deemed to have discovered a data breach when the contractor knew or should have known of breach of such information. When a business associate is part of VHA contract, notification to the covered entity (VHA) shall be made in accordance with the executed BAA.
- 16.8.** If the Contractor or any of its agents fails to protect VA sensitive personal information or otherwise engages in conduct which results in a data breach involving any VA sensitive personal information the contractor/subcontractor processes or maintains under the contract; the contractor shall pay liquidated damages to the VA as set forth in clause [852.211-76, Liquidated Damages— Reimbursement for Data Breach Costs](#).
- 16.9.** BAA is not required.

17. RECORDS MANAGEMENT OBLIGATIONS

A. Applicability

This clause applies to all Contractors whose employees create, work with, or otherwise handle Federal records, as defined in Section B, regardless of the medium in which the record exists.

B. Definitions

“Federal record” as defined in 44 U.S.C. § 3301, includes all recorded information, regardless of form or characteristics, made or received by a Federal agency under Federal law or in connection with the transaction of public business and preserved or appropriate for preservation by that agency or its legitimate successor as evidence of the organization, functions, policies, decisions, procedures, operations, or other activities of the United States Government or because of the informational value of data in them.

The term Federal record:

1. includes VHA records.
2. does not include personal materials.
3. applies to records created, received, or maintained by Contractors pursuant to their VHA contract.
4. may include deliverables and documentation associated with deliverables.

C. Requirements

1. Contractor shall comply with all applicable records management laws and regulations, as well as National Archives and Records Administration (NARA) records policies, including but not limited to the Federal Records Act (44 U.S.C. chs. 21, 29, 31, 33), NARA regulations at 36 CFR Chapter XII Subchapter B, and those policies associated with the safeguarding of records covered by the Privacy Act of 1974 (5 U.S.C. 552a). These policies include the preservation of all records, regardless of form or characteristics, mode of transmission, or state of completion.
2. In accordance with 36 CFR 1222.32, all data created for Government use and delivered to, or falling under the legal control of, the Government are Federal records subject to the provisions of 44 U.S.C. chapters 21, 29, 31, and 33, the Freedom of Information Act (FOIA) (5 U.S.C. 552), as amended, and the Privacy Act of 1974 (5 U.S.C. 552a), as amended and must be managed and scheduled for disposition only as permitted by statute or regulation.
3. In accordance with 36 CFR 1222.32, the Contractor shall maintain all records created for Government use or created in the course of performing the contract and/or delivered to, or under the legal control of the Government and must be managed in accordance with Federal law. Electronic records and associated metadata must be accompanied by

sufficient technical documentation to permit understanding and use of the records and data.

4. VHA and its contractors are responsible for preventing the alienation or unauthorized destruction of records, including all forms of mutilation. Records may not be removed from the legal custody of VHA or destroyed except in accordance with the provisions of the agency records schedules and with the written concurrence of the Head of the Contracting Activity. Willful and unlawful destruction, damage, or alienation of Federal records is subject to the fines and penalties imposed by 18 U.S.C. 2701. In the event of any unlawful or accidental removal, defacing, alteration, or destruction of records, the Contractor must report to VHA. The agency must report promptly to NARA in accordance with 36 CFR 1230.
5. The Contractor shall immediately notify the appropriate Contracting Officer upon discovery of any inadvertent or unauthorized disclosures of information, data, documentary materials, records, or equipment. Disclosure of non-public information is limited to authorized personnel with a need-to-know as described in the [contract vehicle]. The Contractor shall ensure that the appropriate personnel, administrative, technical, and physical safeguards are established to ensure the security and confidentiality of this information, data, documentary material, records, and/or equipment is properly protected. The Contractor shall not remove material from Government facilities or systems, or facilities or systems operated or maintained on the Government's behalf, without the express written permission of the Head of the Contracting Activity. When information, data, documentary material, records, and/or equipment is no longer required, it shall be returned to VHA control or the Contractor must hold it until otherwise directed. Items returned to the Government shall be hand-carried, mailed, emailed, or securely electronically transmitted to the Contracting Officer or address prescribed in the [contract vehicle]. Destruction of records is EXPRESSLY PROHIBITED unless in accordance with Paragraph (4).
6. The Contractor is required to obtain the Contracting Officer's approval prior to engaging in any contractual relationship (sub-contractor) in support of this contract requiring the disclosure of information, documentary material, and/or records generated under, or relating to, contracts. The Contractor (and any sub-contractor) is required to abide by Government and VHA guidance for protecting sensitive, proprietary information, classified, and controlled unclassified information.
7. The Contractor shall only use Government IT equipment for purposes specifically tied to or authorized by the contract and in accordance with VHA policy.
8. The Contractor shall not create or maintain any records containing any non-public VHA information that is not specifically tied to or authorized by the contract.
9. The Contractor shall not retain, use, sell, or disseminate copies of any deliverable that contains information covered by the Privacy Act of 1974 or that which is generally protected from public disclosure by an exemption to the Freedom of Information Act.
10. The VHA owns the rights to all data and records produced as part of this contract. All deliverables under the contract are the property of the U.S. Government for which VHA shall have unlimited rights to use, dispose of, or disclose such data contained therein as it

determines to be in the public interest. Any Contractor rights in the data or deliverables must be identified as required by FAR 52.227-11 through FAR 52.227-20.

11. Training. All Contractor employees assigned to this contract who create, work with or otherwise handle records are required to take VHA-provided records management training. The Contractor is responsible for confirming training has been completed according to agency policies, including initial training and any annual or refresher training.

[Note: To the extent an agency requires contractors to complete records management training, the agency must provide the training to the contractor.]

D. *Flow down of requirements to subcontractors*

1. The Contractor shall incorporate the substance of this clause, its terms, and requirements including this paragraph, in all subcontracts under this [contract vehicle], and require written subcontractor acknowledgment of same.
2. Violation by a subcontractor of any provision set forth in this clause will be attributed to the Contractor.